LIBBY — STANDARD OF CARE

urothelial-mets-her2-discordant-kndl

Standard of care options — approved and guideline-endorsed strategies

Generated 2026-08-20

Libby is an experimental decision-support tool. The recommendations in this report have not been reviewed by a clinician treating this patient. Do not act on this report without consulting a qualified oncologist.

Standard of care options

The treatment strategies that are standard for this patient's situation, meaning a regulator approved them for a population that includes this patient or a major academic or clinical-society guideline carries them. This report runs alongside the targetable-feature ranking rather than in place of it, and it does not narrow what that ranking surfaces.

Assessed: 12 standard options. **Consider now:** 3. **Behind an open gate:** 3. **Already received:** 3.

Twelve standard options were assessed. Three are open now, three sit behind a gate a records request or a pending stain can close, three have already been used, and three were screened and set aside. The one that matters most is platinum: both NCCN and ESMO carry platinum-based chemotherapy as the standard next systemic line after progression on enfortumab vedotin plus pembrolizumab, and this patient has never received it. No platinum appears anywhere in the treatment history, which makes a full guideline-preferred line still unspent. Whether that means cisplatin or carboplatin turns on an eGFR the record does not contain and on how much added neuropathy the team will accept on top of established CIPN.

Already spent: the preferred first line itself (EV plus pembrolizumab, a 25-month complete response), pembrolizumab as a single agent (progression happened on maintenance, so the TMB-H label does not reopen it), and SBRT to both known nodal sites in June 2026.

Gated: continuation of T-DXd, the current therapy, waits on the validated 2026-08 HER2 re-stain, already drawn, tier unretrieved. Erdafitinib waits on FGFR3 status sitting in the same unretrieved sequencing reports. EV rechallenge waits on contemporary Nectin-4 expression and on the neuropathy that ended the first course.

This report adds the standard-of-care options alongside the experimental ranking; it does not remove, reorder, or argue against anything that ranking surfaces, and the one bridge between the two tables is the sequencing section below, which names where the tracks compete or foreclose one another without choosing between them.

Options to consider now

Standard options this patient is eligible for, with an endorsement that covers their situation, and no unresolved gate in front of them.

Platinum-based chemotherapy (gemcitabine plus cisplatin, or gemcitabine plus carboplatin)

Priority: essential **Modality:** Systemic chemotherapy **Line:** Second line **Intent:** life-prolonging intent

What makes it standard.

- NCCN subsequent-line systemic therapy for platinum-naive patients, category 2A (covers this patient · NCCN Bladder Cancer v1.2026)
    - covers: Locally advanced or metastatic urothelial carcinoma progressing after prior systemic therapy in patients who have not previously received platinum-based chemotherapy: gemcitabine plus cisplatin (cisplatin-eligible) or gemcitabine plus carboplatin (cisplatin-ineligible).
- ESMO recommended after progression on first-line enfortumab vedotin plus pembrolizumab (interim update)

(covers this patient · ESMO Clinical Practice Guideline interim update, Ann Oncol 2024)
    - covers: After progression on first-line enfortumab vedotin plus pembrolizumab, standard platinum-based chemotherapy, without maintenance avelumab, in unselected patients.

Fit to this patient. Conditionally eligible Platinum-naive per prior_therapies[], ECOG 1, hematology and hepatic labs adequate. Creatinine/eGFR is absent from the record, so cisplatin eligibility cannot be judged; carboplatin is the guideline-recognized fallback.

- blocker: creatinine/eGFR not in the available record (history of obstructive hydroureteronephrosis, stented 2023-11, resolved 2024); cisplatin dosing cannot be judged without it
- blocker: chronic CIPN from enfortumab vedotin raises the cost of cisplatin neurotoxicity; carboplatin substitution avoids most of it

The single most consequential eligibility fact in this case: platinum-based chemotherapy has never been given, and both NCCN and ESMO carry it as the standard next systemic line after progression on EV plus pembrolizumab. Post-EV+P platinum has no dedicated phase 3, so the endorsements rest on the historical first-line doublet data plus guideline consensus. Cisplatin vs carboplatin turns on the missing eGFR and on how much neuropathy risk the team will accept; the checkpoint-containing first-line triplets do not apply after progression on pembrolizumab.

Key evidence.

- von der Maase 2000 (GC vs MVAC) · randomized phase 3 · n=405 · median OS 13.8 mo (GC) vs 14.8 mo (MVAC), HR 1.04 (95% CI 0.82-1.32); similar efficacy with less toxicity for GC
- De Santis 2012 (EORTC 30986, gem-carbo) · randomized phase 2/3, cisplatin-unfit · n=238 · median OS 9.3 mo (gemcitabine-carboplatin) vs 8.1 mo (M-CAVI); ORR 41.2% vs 30.3%

Toxicities that would change the decision. cisplatin neurotoxicity would stack on established CIPN (a stated veto); carboplatin largely avoids it; high emetogenicity, against active severe nausea on T-DXd; nephrotoxicity risk unquantifiable until eGFR is retrieved; cytopenias (current counts adequate: Hgb 13.8, PLT 170)

Alongside the targeted options. Competes for the same line of therapy. Occupies the same next-line slot as the ranked investigational options, with washout implications for their enrollment windows. Sequencing also runs the other way: at least one ranked protocol (sacituzumab tirumotecan) requires prior platinum, so this regimen is currently the only route to that eligibility.

Last verified. 2026-08-20

References. guideline:NCCN Bladder Cancer v1.2026; pmid:38490358; pmid:11001674; pmid:22162575

Structured surveillance after consolidative SBRT (imaging plus serial ctDNA)

Priority: medium **Modality:** Surveillance **Line:** Not line-indexed **Intent:** disease-control intent

What makes it standard.

- NCCN follow-up recommendations, metastatic disease (partial population match · NCCN Bladder Cancer v1.2026)
    - covers: Post-treatment surveillance for metastatic urothelial carcinoma: cross-sectional imaging of chest, abdomen, and pelvis at regular intervals, with labs and symptom-directed workup. Written as follow-up

scheduling rather than as an explicit alternative to continuing systemic therapy.

Fit to this patient. Eligible All known disease was treated with SBRT 2026-06, two subsequent Signatera draws read 0 MTM/mL, and two tumor-informed ctDNA programs plus regular imaging are already running. Nothing in the profile blocks a monitored break.

With all known disease irradiated and the T-DXd stop/continue decision under active review, surveillance is a real management strategy rather than a default, and the infrastructure for it (serial dual-platform ctDNA, q3-4wk labs, imaging) is already in place. The guideline carries surveillance scheduling for treated metastatic disease rather than an explicit stop-therapy arm, so the endorsement is a partial match. The discordant ctDNA platforms cut both ways and the next serial draws are the deciding input.

Toxicities that would change the decision. no treatment toxicity; the exposure is progression between assessments; the two zero ctDNA draws carry a logged internal conflict (also listed as pending in one table) and NeXT Personal remains positive at 813 PPM, so molecular response is not yet confirmed

Alongside the targeted options. Competes for the same line of therapy. A monitored break occupies the same decision slot as every systemic option on either table, and it interacts with trial screening in a specific way: SBRT has already consolidated the only known measurable sites, so the RECIST-measurability question the ranked trials share is live regardless of whether therapy continues.

Last verified. 2026-08-20

References. guideline:NCCN Bladder Cancer v1.2026

Intravesical BCG maintenance (bladder-directed)

Priority: low **Modality:** Locoregional therapy **Line:** Not line-indexed **Intent:** risk reduction

What makes it standard.

- AUA AUA/SUO non-muscle-invasive bladder cancer guideline: maintenance BCG for high-risk NMIBC (partial population match · AUA/SUO NMIBC guideline, 2024 amendment)
    - covers: Maintenance BCG (SWOG 3-week schedule at 3, 6, 12, 18, 24, 30, 36 months) for high-grade T1 and other high-risk non-muscle-invasive disease after induction response. Written for NMIBC without concurrent metastatic disease.
- FDA on-label (TICE BCG) (partial population match · FDA label (TICE BCG))
    - covers: Intravesical BCG for carcinoma in situ and for prophylaxis of recurrent Ta/T1 papillary tumors of the bladder.

Fit to this patient. Likely eligible Induction x6 (2025-04/05) and one maintenance course (2025-11) were tolerated and the bladder has stayed endoscopically clear since; the scheduled 2026 maintenance course was not given. Nothing in the profile contraindicates further instillation.

Prior exposure. Induction BCG x6 in 2025-04/05 for high-grade T1 intravesical recurrence, best response CR, followed by maintenance x3 in 2025-11; the subsequent 2026 maintenance course was not given. Not stopped for toxicity; the record does not state why the course lapsed.

Included because a reader tracking the SWOG schedule would notice the 2026 maintenance course is missing. The guideline carriage is written for NMIBC without concurrent metastatic disease, so whether continued maintenance still earns its place is a urology-team call weighed against the systemic picture, not a default.

Key evidence.

- SWOG 8507 (Lamm 2000) · randomized, maintenance vs no maintenance · n=384 · median RFS 76.8 vs 35.7 months with maintenance

Toxicities that would change the decision. cystitis-type irritative symptoms; granulomatous inflammation (already confounded one FAPi PET in this case)

Alongside the targeted options. Independent of the targeted options. Bladder-directed therapy does not consume a systemic line or interact with trial washouts; its one case-specific interaction is diagnostic, since BCG granulomas are a known false-positive source on FAP-directed imaging.

Last verified. 2026-08-20

References. guideline:AUA/SUO NMIBC guideline (2024 amendment); fda:TICE BCG package insert; pmid:10737480

Standard options behind an open gate

Endorsed for this patient's situation, but gated on a result that is missing, pending, or below threshold. The gate has to close before the option is real.

Erdafitinib (conditional on FGFR3 alteration)

Priority: high **Modality:** Targeted therapy **Line:** Second line **Intent:** life-prolonging intent

What makes it standard.

- FDA full approval (converted from accelerated, 2024-01-19) (partial population match · FDA full approval 2024-01-19)
    - covers: Adult patients with locally advanced or metastatic urothelial carcinoma with susceptible FGFR3 genetic alterations, as determined by an FDA-approved companion diagnostic, whose disease has progressed on or after at least one line of prior systemic therapy. Not recommended for patients who are eligible for and have not received prior PD-1/PD-L1 inhibitor therapy.
- ESMO recommended in selected FGFR-altered tumors after progression on enfortumab vedotin plus pembrolizumab (interim update) (partial population match · ESMO Clinical Practice Guideline interim update, Ann Oncol 2024)
    - covers: Erdafitinib in selected FGFR-altered tumors as an option after progression on first-line enfortumab vedotin plus pembrolizumab.
- NCCN subsequent-line systemic therapy for FGFR3-altered disease, category 2A (partial population match · NCCN Bladder Cancer v1.2026)
    - covers: Erdafitinib for locally advanced or metastatic urothelial carcinoma with susceptible FGFR3 alterations after prior systemic therapy.

Fit to this patient. Not determinable from the record The gating fact is missing, not negative: FGFR3 status appears nowhere in the derived record, even though the 2023 tissue NGS was run and counts exactly one OncoKB-actionable alteration it never names. Line and prior PD-1 requirements are met.

Biomarker gate. FGFR3 susceptible alteration susceptible FGFR3 alteration by an FDA-approved companion diagnostic (found in roughly 15-20% of metastatic urothelial carcinoma) (status unknown)

The one approved oral targeted option in this disease, with a phase 3 OS benefit in exactly this position (post anti-PD-1), sitting behind a records request rather than a new test. Every endorsement is written for FGFR3-altered disease, and FGFR3 status is simply unretrieved: the 2023 NGS report names one OncoKB-actionable alteration without saying which. A laboratory-developed panel result may still need companion-diagnostic confirmation before dosing.

Key evidence.

- THOR cohort 1 · randomized phase 3 vs chemotherapy, post anti-PD-(L)1 · n=266 · median OS 12.1 vs 7.8 mo, HR 0.64 (95% CI 0.47-0.88)

Toxicities that would change the decision. hyperphosphatemia and central serous retinopathy require phosphate monitoring and serial eye exams; stomatitis, nail and skin changes, diarrhea; does not touch the four stated vetoes (no neuropathy, low emetogenicity, no ILD signal, no pancreatitis signal)

Alongside the targeted options. Competes for the same line of therapy. Shares its FGFR3 gate with the ranked investigational FGFR3 option (vepugratinib, rank 7) and with the rank-1 records-retrieval workup that closes the gate for both; if FGFR3 confirms, the approved drug and the trial option compete for the same slot.

Last verified. 2026-08-20

References. fda:Balversa USPI (2024); pmid:38490358; guideline:NCCN Bladder Cancer v1.2026; pmid:37870920; nct:NCT03390504; pmid:31340094

Trastuzumab deruxtecan continuation (current therapy)

Priority: high **Modality:** Targeted therapy **Line:** Second line **Intent:** disease-control intent

What makes it standard.

- FDA tumor-agnostic accelerated approval (covers this patient · FDA accelerated approval 2024-04-05)
  - covers: Adult patients with unresectable or metastatic HER2-positive (IHC 3+) solid tumors who have received prior systemic treatment and have no satisfactory alternative treatment options.
- NCCN biomarker-directed therapy, HER2 IHC 3+, category 2A (default-category convention) (covers this patient · NCCN Bladder Cancer v1.2026)
  - covers: Trastuzumab deruxtecan for HER2 IHC 3+ urothelial carcinoma in the biomarker-directed systemic therapy table, regardless of previous therapy.

Fit to this patient. Eligible Currently receiving (2 doses since 2026-07). The tumor-agnostic label is met on three concordant validated IHC 3+ results (2023-2025); the open question is whether the current tumor retains that expression.

Biomarker gate. HER2 (contemporary validated IHC on current tumor tissue) IHC 3+ (reflex ISH if 2+); historical 3+ satisfies the label, but the continuation decision is gated on the 2026-08 re-stain (result pending)

The patient's active therapy, on-label under the tumor-agnostic HER2 IHC 3+ approval, and the continuation position all four board personas ranked first. Continuation is not yet a settled call: the validated 2026-08 re-stain (drawn, tier unretrieved) decides whether the current tumor still meets the 3+ bar, and tolerability plus ctDNA kinetics are explicit open decision inputs. If the contemporary tier comes back low, the on-label footing narrows to a cross-tumor extrapolation and the calculus changes.

Key evidence.

- DESTINY-PanTumor02 · open-label phase 2 basket · n=267 · ORR 61.3% in central IHC 3+ vs 37.1% overall; ILD/pneumonitis 10.5% including 3 deaths

Toxicities that would change the decision. ILD/pneumonitis risk against the unresolved 2026-03 vs 2026-06 chest CT discrepancy (a stated veto category); severe nausea and bloating after dose 2 is an active, dose-limiting problem (GI veto engaged); cytopenias and fatigue with cumulative dosing

Alongside the targeted options. May foreclose a targeted option. Continuing consumes the washout window the ranked HER2 trial options need, and accumulating T-DXd exposure can itself exclude enrollment in some HER2-directed protocols; stopping opens those doors. The stop/continue call is this row's pending HER2 gate plus ctDNA kinetics.

Last verified. 2026-08-20

References. fda:Enhertu USPI tumor-agnostic indication (2024); guideline:NCCN Bladder Cancer v1.2026; pmid:37870536; nct:NCT04482309

Enfortumab vedotin rechallenge (monotherapy)

Priority: medium **Modality:** Targeted therapy **Line:** Third line or later **Intent:** disease-control intent

What makes it standard.

- FDA full approval (monotherapy, 2021-07) (partial population match · FDA full approval 2021-07)
    - covers: Adult patients with locally advanced or metastatic urothelial cancer who have previously received a PD-1 or PD-L1 inhibitor and platinum-containing chemotherapy, or are ineligible for cisplatin-containing chemotherapy and have previously received one or more prior lines of therapy.
- NCCN subsequent-line systemic therapy option, category 2A (partial population match · NCCN Bladder Cancer v1.2026)
    - covers: Enfortumab vedotin monotherapy for locally advanced or metastatic urothelial carcinoma after prior systemic therapy; rechallenge after first-line EV plus pembrolizumab discontinued for toxicity is not separately addressed.

Fit to this patient. Conditionally eligible The label population is platinum-experienced or cisplatin-ineligible; this patient is platinum-naive and cisplatin eligibility is undetermined (no eGFR on file), so the on-label fit is partial. The stronger conditions are clinical: the drug was stopped for neuropathy, and current target expression is unverified.

- blocker: chronic CIPN from the prior EV course, with repeated dose reductions, is the reason the drug stopped and would be re-provoked by the MMAE payload
- blocker: Nectin-4 expression after ~25 months of EV is unknown; antigen downregulation is a recognized EV-resistance mechanism
- blocker: label population (prior platinum, or cisplatin-ineligible) is not established for this patient

Prior exposure. Received as part of first-line EV plus pembrolizumab for ~25 months (best response CR, liver metastases resolved). EV was repeatedly dose-reduced for peripheral neuropathy and discontinued 2025-12 for CIPN, not progression; nodal progression came later, on pembrolizumab maintenance.

Biomarker gate. Nectin-4 (contemporary expression after prolonged EV exposure) no label expression threshold; the case's own decision rule gates re-targeting on a contemporary re-stain or the 2026-06 Nectin-4 imaging read (result pending)

The rechallenge question is legitimate because the drug produced a 25-month CR and stopped for toxicity rather than progression, which is the more favorable rechallenge setting. It is still a gated extrapolation: no approval or guideline text covers rechallenge after first-line EV plus pembrolizumab, current Nectin-4 expression is unverified, and the neuropathy that forced discontinuation has not resolved. The 2026-06 Nectin-4 imaging read or a contemporary stain is the decision-relevant test before any re-exposure discussion.

Key evidence.

- EV-301 · randomized phase 3 vs chemotherapy · n=608 · median OS 12.9 vs 9.0 mo, HR 0.70 (95% CI 0.56-0.89)

Toxicities that would change the decision. peripheral neuropathy: the toxicity that ended the first course and the top stated veto; hyperglycemia, against prediabetes on metformin; rash and ocular surface effects

Alongside the targeted options. Competes for the same line of therapy. Sits in the same Nectin-4 axis as the ranked next-generation options (LY4052031 novel-payload ADC, 225Ac-AKY-1189 radioligand), which share the contemporary-expression gate but avoid the MMAE payload; prior EV exposure is an exclusion in some Nectin-4 protocols, so rechallenge and trial entry interact both ways.

Last verified. 2026-08-20

References. fda:Padcev USPI (monotherapy indication); guideline:NCCN Bladder Cancer v1.2026; pmid:33577729; nct:NCT03474107

Already received

Carried so the report reads as a complete account of the standard options rather than a list that quietly omits what has been tried.

Enfortumab vedotin plus pembrolizumab (first line)

Priority: low **Modality:** Targeted therapy **Line:** First line **Intent:** life-prolonging intent

What makes it standard.

- NCCN preferred first-line regimen, category 1 (covers this patient · NCCN Bladder Cancer v1.2026)
    - covers: First-line systemic therapy for locally advanced or metastatic urothelial carcinoma, cisplatin-eligible and cisplatin-ineligible alike.
- ESMO preferred first-line therapy irrespective of platinum eligibility [I, A] (interim update) (covers this patient · ESMO Clinical Practice Guideline interim update, Ann Oncol 2024)
    - covers: Enfortumab vedotin plus pembrolizumab as preferred first-line therapy for advanced or metastatic urothelial carcinoma.

- FDA approval 2023-12-15 (covers this patient · FDA approval 2023-12-15)
  - covers: Enfortumab vedotin with pembrolizumab for adult patients with locally advanced or metastatic urothelial cancer.

Fit to this patient. Already received First-line therapy from late 2023; the standard-of-care question at this line is spent.

Prior exposure. Given first line for ~25 months with best response CR (liver metastases resolved by 2024-08). EV was repeatedly dose-reduced for peripheral neuropathy and stopped 2025-12 for CIPN, not progression; pembrolizumab continued as maintenance until nodal progression and ctDNA rise in 2026.

Carried so the table reads as a complete account: the guideline-preferred first-line standard was given, worked for 25 months, and its two components stopped for different reasons (EV for toxicity, pembrolizumab for progression). That split is why EV rechallenge and pembrolizumab appear as separate rows with different verdicts.

Key evidence.

- EV-302 · randomized phase 3 vs platinum-based chemotherapy · n=886 · median OS 31.5 vs 16.1 mo, HR 0.47 (95% CI 0.38-0.58)

Toxicities that would change the decision. left this patient with chronic CIPN (dose reductions, then discontinuation) and a recovered episode of asymptomatic pancreatitis

Last verified. 2026-08-20

References. guideline:NCCN Bladder Cancer v1.2026; pmid:38490358; pmid:38446675; nct:NCT04223856

Pembrolizumab (TMB-H tumor-agnostic footing)

Priority: low **Modality:** Immunotherapy **Line:** Maintenance **Intent:** life-prolonging intent

What makes it standard.

- FDA tumor-agnostic accelerated approval (TMB-H) (partial population match · FDA accelerated approval 2020-06)
  - covers: Adult and pediatric patients with unresectable or metastatic tumor mutational burden-high (TMB-H, at least 10 mutations/megabase) solid tumors, as determined by an FDA-approved test, that have progressed following prior treatment and who have no satisfactory alternative treatment options.
- NCCN preferred subsequent-line therapy post-platinum, category 1 (different population · NCCN Bladder Cancer v1.2026)
  - covers: Pembrolizumab for locally advanced or metastatic urothelial carcinoma after platinum-based chemotherapy (KEYNOTE-045 population).

Fit to this patient. Already received Progression occurred on this drug: nodal progression and ctDNA rise developed during pembrolizumab maintenance, which was then held 2026-07.

Prior exposure. Given within first-line EV plus pembrolizumab (best response CR), then continued as maintenance monotherapy 2025-12 to 2026-07. Best response on maintenance was PD: biopsy-proven nodal progression with ctDNA rise on drug. Held 2026-07, not stopped for toxicity.

Biomarker gate. TMB at least 10 mut/Mb by an FDA-approved test 12.4 mut/Mb on file, but measured on an institutional 468-gene panel rather than the approved companion assay (status unknown)

Listed because the TMB-H tumor-agnostic label looks applicable on paper and is not: the patient progressed on pembrolizumab itself, the TMB value comes from an assay the indication does not name, and the ESMO interim update explicitly discourages single-agent checkpoint rechallenge after EV plus pembrolizumab without further evidence. Checkpoint re-intensification (adding CTLA-4) is a different question and lives in the experimental table at rank 10.

Key evidence.

- KEYNOTE-158 (TMB-H cohort) · phase 2 basket, retrospective TMB analysis · n=790 · ORR 29% (95% CI 21-39) in TMB-H vs 6% (95% CI 5-8) in non-TMB-H

Toxicities that would change the decision. prior irAE surveillance already includes q3-4wk amylase/lipase after the 2025 pancreatitis episode

Last verified. 2026-08-20

References. fda:Keytruda USPI TMB-H indication; guideline:NCCN Bladder Cancer v1.2026; pmid:32919526; pmid:38490358

SBRT to oligometastatic nodal disease

Priority: low **Modality:** Radiotherapy **Line:** Not line-indexed **Intent:** disease-control intent

What makes it standard.

- NCCN radiation therapy for selected metastatic sites (principles of radiation therapy; useful in certain circumstances) (partial population match · NCCN Bladder Cancer v1.2026)
  - covers: Metastasis-directed or palliative radiotherapy for selected patients with metastatic urothelial carcinoma; oligometastatic consolidation is carried as a selective option rather than a categorical recommendation.

Fit to this patient. Already received Delivered 2026-06 to the known oligometastatic nodal sites.

Prior exposure. SBRT 2026-06 to the biopsy-proven left para-aortic node and the probable perivesical node, the only known active sites. Given for oligometastatic consolidation, not stopped; formal response assessment is pending, and two subsequent Signatera draws read 0 MTM/mL.

Already delivered to both known sites, so it appears here as record rather than choice. Repeat SBRT at a future oligoprogression event would be a standard consideration under the same selective guideline carriage, with the randomized support coming from mixed-primary oligometastatic data rather than urothelial-specific trials.

Key evidence.

- SABR-COMET · randomized phase 2, mixed primaries · n=99 · median OS 41 vs 28 mo, HR 0.57 (95% CI 0.30-1.10); supports the strategy class, not this histology specifically

Toxicities that would change the decision. delivered without recorded acute toxicity; late nodal-field effects pending

Alongside the targeted options. May foreclose a targeted option. By consolidating the only known measurable sites, the June SBRT is what created the RECIST-measurability question that recurs across the ranked trials' screening gates; any future repeat SBRT at oligoprogression would re-open the same trade-off between local control and trial-evaluable disease.

Last verified. 2026-08-20

References. guideline:NCCN Bladder Cancer v1.2026; pmid:30982687; nct:NCT01446744

Assessed and set aside

Screened against this patient and found not to apply. Listed for the audit trail.

Avelumab switch maintenance after platinum

Priority: low **Modality:** Immunotherapy **Line:** Maintenance **Intent:** life-prolonging intent

What makes it standard.

- FDA approval 2020-06 (maintenance) (different population · FDA approval 2020-06)
    - covers: Maintenance treatment of patients with locally advanced or metastatic urothelial carcinoma that has not progressed with first-line platinum-containing chemotherapy.
- ESMO interim update: platinum after EV plus pembrolizumab progression is recommended without maintenance avelumab (different population · ESMO Clinical Practice Guideline interim update, Ann Oncol 2024)
    - covers: JAVELIN Bladder 100 studied checkpoint-naive patients after first-line platinum; the interim update recommends platinum-based chemotherapy without maintenance avelumab when platinum follows progression on enfortumab vedotin plus pembrolizumab.

Fit to this patient. Likely ineligible The indication is written for checkpoint-naive patients whose disease has not progressed on first-line platinum; this patient progressed on PD-1 maintenance, and any future platinum here would be later-line.

- blocker: progression occurred on pembrolizumab maintenance, so switch maintenance with another PD-(L)1 antibody repeats a failed axis
- blocker: ESMO interim update names this exact sequence and recommends platinum without maintenance avelumab

Set aside so the platinum row is not misread as importing the whole first-line sequence: if platinum is given here it comes after PD-1 progression, a setting where the avelumab maintenance evidence does not apply and the ESMO interim update explicitly recommends platinum without it.

Key evidence.

- JAVELIN Bladder 100 · randomized phase 3, maintenance vs best supportive care · n=700 · median OS 21.4 vs 14.3 mo, HR 0.69 (95% CI 0.56-0.86)

Last verified. 2026-08-20

References. fda:Bavencio USPI (maintenance indication); pmid:38490358; pmid:32945632; nct:NCT02603432

Sacituzumab govitecan (mUC approval withdrawn)

Priority: low **Modality:** Targeted therapy **Line:** Third line or later **Intent:** life-prolonging intent

What makes it standard.

- FDA accelerated approval 2021-04, voluntarily withdrawn 2024 after the confirmatory trial (different population · FDA accelerated approval 2021-04; withdrawal announced 2024)
    - covers: Was: locally advanced or metastatic urothelial cancer after platinum-containing chemotherapy and a PD-1/PD-L1 inhibitor. The confirmatory TROPiCS-04 trial did not verify benefit and the mUC indication was withdrawn; guideline listings followed.

Fit to this patient. Likely ineligible No current approval or guideline carriage in urothelial carcinoma, and the withdrawn indication's population (post-platinum) would not have covered this platinum-naive patient anyway.

- blocker: mUC accelerated approval withdrawn after TROPiCS-04; no guideline carriage remains
- blocker: withdrawn label population required prior platinum, which this patient has not had

Listed because a TROP2-low-positive readout makes this the drug a reader would go looking for: it is no longer standard of care in urothelial carcinoma after the 2024 withdrawal. TROP2-directed options for this case are investigational and already ranked in the experimental table.

Last verified. 2026-08-20

References. fda:Trodelvy mUC indication withdrawal (2024)

Single-agent taxane (paclitaxel or docetaxel)

Priority: low **Modality:** Systemic chemotherapy **Line:** Third line or later **Intent:** life-prolonging intent

What makes it standard.

- NCCN other recommended / useful-in-certain-circumstances subsequent-line single agents, category 2A (covers this patient · NCCN Bladder Cancer v1.2026)
    - covers: Single-agent paclitaxel or docetaxel among later-line options for locally advanced or metastatic urothelial carcinoma after prior systemic therapy.

Fit to this patient. Likely ineligible Established chronic CIPN from enfortumab vedotin, severe enough to force dose reductions and discontinuation and to sustain a dedicated management project, makes a neurotoxic taxane a poor physiologic fit independent of preference.

- blocker: chronic CIPN: taxane neurotoxicity would compound the same injury that ended EV

Guideline-carried for this line but set aside on the profile: single-agent taxanes offer modest response rates and their defining toxicity lands directly on this patient's chronic CIPN, while a platinum doublet remains unused ahead of them in the same guideline.

Toxicities that would change the decision. peripheral neuropathy is the class toxicity and the patient's most established treatment injury

Last verified. 2026-08-20

References. guideline:NCCN Bladder Cancer v1.2026